



This policy is currently under review. It remains in effect until the review is completed and the policy is updated.

NMHS POLICY

Management of 'Did Not Wait' and 'Left at Own Risk' Patients at Emergency Department/Centre

Scope (Staff)	All NMHS staff
Scope (Area)	Emergency Department/Centre at NMHS sites and services
Summary	This policy aims to mitigate risk for patients who 'Did Not Wait' or 'Left at Own Risk' at Emergency Department/Centre by outlining the process for the management of these patients as well as preventative measures. This policy does not apply to admitted patients who 'Discharge Against Medical Advice' (DAMA). Refer to relevant site policy and procedures for further information related to DAMA.
Approval	North Executive Team
Sponsor	Area Director Clinical Services
Contact	Senior Policy and Compliance Officer, Consumer Experience and Clinical Excellence
Version	1.0
Next Review Date	16/01/2025
NSQHS Standards	Communicating for Safety (Std 6) Partnering with Consumers (Std 2) Recognising and Responding to Acute Deterioration (Std 8)
ISD Number	2742
Status	CURRENT

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1. Aim

North Metropolitan Health Service (NMHS) is committed to ensuring the provision of safe and culturally appropriate care for all patients attending Emergency Departments (ED)/Emergency Centre (EC) at NMHS sites and services. This policy aims to mitigate risk for patients who 'Did Not Wait' or 'Left at Own Risk' at NMHS ED/EC by outlining the process for the management of these patients as well as preventative measures.

This policy **does not** apply to admitted patients who 'Discharge Against Medical Advice' (DAMA). Refer to relevant site policies and procedures for further information related to DAMA.

2. Background

Patients may present to ED/EC at NMHS sites and services and leave after triage without waiting to be assessed and/or treated.

NMHS encourages and supports patients to participate fully in decision-making related to their care and acknowledges that this may mean they choose to leave without full assessment and/or treatment after being triaged at NMHS ED/EC. However, a patient's decision to leave before being fully assessed and/or treated does not rule out the presence of serious pathology. .

DNW/LAOR is a Key Performance Indicator (KPI) within the Health Service Performance Report (HSPR). Refer to [WA Health Patient Activity Data Policy \(MP 0164/21\)](#) and [Emergency Department Patient Activity Data Business Rules](#).

3. Risk

Non-compliance with this policy may result in a breach of mandatory requirements, harm to patients and/or staff, KPI variation, and/or reputational and image damage to NMHS.

4. Policy Principles

NMHS is committed to ensuring that access to sites and services are culturally safe to improve patient outcomes and reduce health disparities. NMHS encourages the use of supportive services available to help meet the individual patient's psychosocial, cultural and spiritual needs throughout their care journey including their presentation at NMHS ED/EC.

For patients who present with limited English language proficiency, language services must be used in accordance with the [WA Health Language Services Policy \(MP 0051/17\)](#). Refer to local policy or procedure on language services on how to arrange an interpreter.

4.1 Prevention of DNW/LAOR

NMHS acknowledges that long wait time may contribute to DNW/LAOR in some cases and is committed to make system improvements to ensure wait time is within the [Australian Triage Scale recommendations](#).

Preventative strategies for DNW/LAOR in all patient cohorts may include open communication to inform the wait time at ED/EC, display of information that acknowledges the wait time and encourages patients to wait, as well as information advising patients to inform a staff member if they choose not to wait. If the patient chooses not to wait, written information should be provided to patient on any alternative services available (e.g., After-hour clinics, Urgent Care Centres), and this is to be documented in the patient's healthcare record.

Available evidence suggests that cultural support provided by Aboriginal workforce in the Emergency Department setting may reduce the risk of DNW/LAOR among Aboriginal people. NMHS strongly recommends the incorporation of Aboriginal employees at ED/EC wherever possible to help build connection and trust, encourage the completion of assessment and treatment, and to communicate with and support Aboriginal people who present at ED/EC in a culturally appropriate and safe way.

4.2 Documentation of DNW and LAOR Patients

In either of the below situations, the patient's absence must be documented on the ED/EC Triage Record and entered in the local Patient Administration Database as 'DNW' or 'LAOR' in line with the definitions, and the Shift Coordinator notified:

- If a triaged patient is in the ED/EC waiting area and then is not found/present when called (3 calls will be attempted); *or*
- If a triaged patient is in an ED/EC bay and then is not found/present when called (3 calls will be attempted).

4.3 Risk assessment of DNW and LAOR Patients

Patients who DNW/LAOR are at risk of deteriorating in the community. A risk assessment must be conducted by the Shift Coordinator in consultation with the Duty Consultant (or delegate) based on information available including the triage score and assessment, any subsequent clinical assessment conducted (if applicable), and historical information available in the local patient administration system.

When conducting a risk assessment, the following factors must be considered wherever applicable:

- The triage score and assessment
- One or more observations that are of clinical concern
- Where the patient is considered in the vulnerable cohort as listed in **section 4.3.1**
- Presentation on more than one occasion during the previous week at any ED/EC
- Previous DNW/LAOR on one or more prior presentations at any ED/EC
- Presentation to triage desk multiple times within a visit

Once the DNW/LAOR patient has been risk assessed, action as outlined in [section 4.4](#) should be undertaken in accordance with site-based escalation processes.

4.3.1 Vulnerable patient cohorts

The following patient cohorts are considered more vulnerable and careful consideration of their individual needs will be applied when conducting the risk assessment:

- Aboriginal people
- Patients of culturally and linguistically diverse (CALD) background
- Patients with limited English language proficiency
- People with disability
- Children and young people (<18 years) (Refer to [WA Health Guidelines for Protecting Children](#))
- People at risk of Family and Domestic Violence (Refer to [NMHS Family and Domestic Violence Framework](#) and [Family and Domestic Violence Hub](#))
- Homeless people
- Patients presenting with mental health complaints (Refer to [section 4.4](#))
- Pregnant women

Refer to available support or specialist services (e.g., Social Work, Aboriginal Hospital Liaison Program, Aboriginal Mental Health Liaison team, Aboriginal Health Practitioner) for guidance if required.

4.4 Management of DNW and LAOR Patients

Based on the risk assessment described in [section 4.3](#), DNW/LAOR patients are categorised into low risk patient, medium risk patient or high risk patient (Refer to [section 5 Definitions](#)). Management of DNW/LAOR patients will be guided by the risk assessment rating.

Available support or specialist services (e.g., Social Work, Aboriginal Mental Health Liaison team) should be requested to ensure the management of vulnerable patient cohorts described above is appropriate and meets their psychosocial, cultural and spiritual needs, where possible.

4.4.1 Low Risk Patients – No Action

Where it is determined that the risk to the patient is low, no further action is required. This decision and the supporting evidence must be documented in the patient's healthcare record.

4.4.2 Moderate Risk Patients – Deferred Action

Where it is determined the risk to the patient is moderate, the patient will be referred for active follow-up via telephone call during business hours. The follow-up may be made by the Shift Coordinator on duty (or delegate) or handed over to the Shift Coordinator of the incoming shift.

For pregnant women >20 weeks' gestation who DNW/LAOR, the management should consider consultation with the obstetric team on call at King Edward Memorial Hospital by the Duty Consultant (or delegate) via the NMHS Switchboard.

All reasonable efforts will be undertaken to make telephone contact with the patient and encourage them to return for assessment and/or treatment. If they prefer not to return, they will be encouraged to see their General Practitioner (GP) as soon as they can. If the patient is unable to be contacted, this must be documented in the patient's healthcare record. On rare occasions, escalation to the 'high risk' may be necessary.

Any decision and subsequent discussions and/or actions must be documented in the patient's healthcare record.

4.4.3 High Risk Patients – Immediate Action

Where it is determined that the risk to the patient is high, the Shift Coordinator (or delegate) will immediately attempt contact with the patient via telephone to encourage their return for assessment and/or treatment.

Where these attempts of contacting the patient are unsuccessful and the risk remains high, the below actions may be undertaken:

- Escalation to senior staff as per site-specific escalation process
- Contact the Next of Kin/Carer (if appropriate)
- In consultation with senior staff, the Shift Coordinator will decide on the necessity for involvement of WA Police.

The decision and any subsequent discussions and/or actions will be documented in the patient's healthcare record.

4.5 Special Considerations for Patients Presenting with Mental Health Complaints

The management of patients presenting with mental health complaints who DNW/LAOR should include consultation with a mental health specialist (e.g., Psychiatric Liaison Nurses, Aboriginal Mental Health Liaison team). Where the patient was referred to the ED/EC by local mental health services, or where a concern regarding the mental state of a patient has been identified, the Shift Coordinator (or delegate) will liaise with the mental health specialist (e.g., Psychiatric Liaison Nurses, Aboriginal Mental Health Liaison team) who may consult with the Community Mental Health Case Manager (if known). Any assessment or follow-up by the mental health specialist (e.g., Psychiatric Liaison Nurses, Aboriginal Mental Health Liaison team) must be documented in the patient's healthcare record.

Any patient referred or detained under the [Mental Health Act 2014](#) for assessment by a psychiatrist who leaves an authorised mental health facility or general hospital (including an ED/EC) without prior authorisation by the treating team must be reported to the Office of the Chief Psychiatrist in accordance with the [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist: Policy for Public Mental Health Services](#).

Patients presenting with deliberate self-harm and/or suicidal ideation will be managed in accordance with the [NMHS Policy Clinical Care of People Who May Be Suicidal](#) and [WA Health Clinical Care of People Who May Be Suicidal Policy \(MP 0074/17\)](#).

5. Definitions

Did not wait (DNW)	Patients presenting to ED/EC who were triaged and registered but did not wait to be assessed and/or treated by a clinician.
Left at own risk (LAOR)	Left at own risk (LAOR) occurs when a non-admitted patient chooses to leave the ED/EC before completion of intended clinical assessment and/or treatment and against the advice of the treating medical practitioner.
Discharged Against Medical Advice (DAMA)	'Discharged Against Medical Advice' (DAMA) occurs when an admitted patient chooses to leave the hospital before the completion of intended treatment and against the advice of the treating clinician. A decision is then made by the patient to discharge against medical advice.
Low risk patient	Refers to DNW/LAOR patients who are at low risk of deteriorating in the community. For example, a patient who is: • able to mobilise independently; • has no intravenous (IV) access; • is not assessed as a danger to self and/or others; • has no identified or anticipated complication relating to the presenting complaint. Note: the listed examples provide a guide only and are not to replace clinical judgement.
Moderate risk patient	Refers to DNW/LAOR patients who are at moderate risk of deteriorating in the community due to risk factors such as: • identified cognitive impairment; • from a vulnerable patient cohort (see section 4.3.1) • has complex and/or unstable chronic diseases or conditions; • has a weakened immune system;

	pregnant women presenting with acute medical/surgical complication. Note: the listed examples provide a guide only and are not to replace clinical judgement.
High risk patient	Refers to DNW/LAOR patients who are at high risk of deteriorating in the community due to risk factors such as: - presented with deliberate self-harm, suicidal ideation or thoughts of harm to self or others; - referred under the Mental Health Act 2014 for assessment by a psychiatrist; - has any invasive devices in situ (excluding pre-existing long-term devices). Note: the listed examples provide a guide only and are not to replace clinical judgement.

6. Compliance and Evaluation

Each NMHS site and service Executive Director will be responsible for ensuring local compliance with this policy.

Data on DNW and LAOR will be captured in HSPR. Each NMHS site and service will be responsible for monitoring their local DNW/LAOR data. Performance concerns related to DNW/LAOR will be escalated in accordance with the [NMHS Area-wide Performance Policy](#).

NMHS Consumer Experience and Clinical Excellence directorate may request ad-hoc audits of healthcare records by sites and services to demonstrate local compliance with this policy.

This policy will be next reviewed 12 months after its publication to ensure incorporation of any updates on NMHS approaches. The scheduled review will draw on available evaluation data during the policy implementation.

7. Document Control

Version	Amendment(s)	TRIM Ref	Published Date
1.0	Published version	D/24/12343	16/01/2024 (Approved 21/12/2023)

Related policies, procedures and guidelines

WA Health/Government Mandatory Policies

- [WA Health Clinical Care of People Who May Be Suicidal Policy \(MP 0074/17\)](#)
- [WA Health Recognising and Responding to Acute Deterioration Policy \(MP 0171/22\)](#)
- [WA Health Patient Activity Data Policy \(MP 0164/21\)](#)
 - [Emergency Department Patient Activity Data Business Rules](#)
- [WA Health Consent to Treatment Policy \(MP 0175/22\)](#)
- [WA Health System Language Services Policy \(MP 0051/17\)](#)
- [WA Health Clinical Incident Management Policy 2019 \(MP 0122/19\)](#)
- [WA Health Aboriginal Health and Wellbeing Policy \(MP 0071/17\)](#)
- [WA Health Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy \(MP 0099/18\)](#)
- [Office of the Chief Psychiatrist Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist: Policy for Public Mental Health Services](#)
- [WA Health Guidelines for Protecting Children](#)

NMHS Policy

[NMHS Area-wide Performance Policy](#)

[NMHS Clinical Care of People Who May Be Suicidal Policy](#)

[NMHS Clinical Incident Management Policy](#)

[NMHS Family Domestic Violence Framework](#)

SCGOPHCG Policy

[SCGOPHCG Discharge Against Medical Advice \(DAMA\) Policy](#)

WNHS Policy

[WNHS Discharge of Patient Policy](#)

[KEMH Obstetrics & Gynaecology Clinical Practice Guideline Discharge of a Patient](#)

References

[*Mental Health Act 2014*](#)

[Australia Commission on Safety and Quality in Health Care: Reducing incomplete treatment in the ED for Aboriginal and Torres Strait Islander patients.](#)

[Preis et. al. 'Dalarinji': A flexible clinic, belonging to and for the Aboriginal people, in an Australian emergency department. Emergency Medicines Australasia. 2022, 34: 46-51.](#)

Useful resources

- [Aboriginal Health Policy Directorate 2018, 'Aboriginal Patient Take Own Leave: Review and recommendations for improvement', Department of Health of Western Australia, Perth.](#)
- [Australian Commission on Safety and Quality in Health Care Recognising and Responding to Acute Deterioration Standard](#)
- [Cultural Security – NMHS Aboriginal Health Hub](#)
- [Family and Domestic Violence Hub](#)

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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